

Health-Related Emoji Coverage as a Standards-Maintenance Issue

Status: Final discussion draft for Microsoft review and referral to the Unicode Technical Committee (UTC) and the Emoji Standard and Research Working Group (ESR).

Executive summary

Health-related emoji coverage is now a coherent standards-maintenance problem. Health concepts are distributed across anatomy, diagnostic, laboratory, medication, emergency-care, and clinical-equipment contexts, while users and implementers encounter them as one practical communication domain. The issue is not that every medical concept should become an emoji. The issue is that the current coverage and review process does not provide a clear way to identify gaps, compare candidates, or coordinate vendor support.

This document asks the UTC and ESR to recognize the maintenance issue, review the organization of the domain, establish prioritization criteria, route individual concepts through the ordinary Emoji proposal process, and clarify how vendors may provide bounded design, accessibility, rights, evidence, and anticipated-implementation support.

No character is requested, approved, or prioritized by this document. Each concept remains subject to the current Unicode Emoji proposal requirements, including public evidence, distinctiveness, exclusion-factor analysis, image rights, and the normal submission form.

Concrete request

Microsoft respectfully asks the UTC to refer this document to the ESR for review and recommendation, with the following five questions before it:

1. **Recognize health-related emoji coverage as a coherent standards-maintenance issue.** The domain should be assessed as a connected coverage problem even when its existing characters remain in different keyboard groups.
2. **Review how the domain is organized.** The review should examine anatomy, diagnostics, laboratory medicine, medication, emergency care, and clinical equipment, including the relationship between existing emoji and missing concepts.
3. **Establish criteria for prioritizing missing concepts.** Criteria should include broad and reproducible usage evidence, distinctiveness, multiple durable meanings, combinability, category completion, non-duplication, visual legibility, accessibility, rights readiness, and implementation feasibility.
4. **Refer individual concepts to the normal Emoji proposal process.** A coverage review must not become a bundled encoding request. Kidney, liver, stomach, EKG/ECG, white blood cell, Pill Box, Inhaler, First Aid Kit, Blood Bag, IV Bag, Leg

Cast, Weight Scale, and any other concept must be evaluated in its own complete submission.

5. **Clarify vendor support.** Vendors may contribute design review, accessibility testing, rights and provenance information, evidence reproducibility, and anticipated implementation planning without asking the Consortium to adopt vendor artwork or pre-commit to encoding.

The requested outcome is procedural clarity and serious technical review, not automatic approval.

Scope of the maintenance review

The review should use a coverage matrix with six domains:

Domain	Existing examples	Gap questions
Anatomy	brain, anatomical heart, lungs, tooth, mechanical limbs	Are major internal organs and body systems represented distinctly enough for ordinary communication?
Diagnostics	x-ray, stethoscope, thermometer-related face	Are common diagnostic modalities and readings represented without over-specific device branding?
Laboratory medicine	test tube, petri dish, DNA, microbe	Are specimen, cell, blood, and laboratory concepts organized coherently?
Medication	pill, syringe, drop of blood	Are medication-use concepts such as Pill Box and Inhaler independently useful and visually distinct?
Emergency care	adhesive bandage, crutch, ambulance, hospital	Are first-response objects and care actions represented as broad paradigms rather than logos?
Clinical equipment	stethoscope, x-ray, crutch	Which equipment concepts have broad public use, a clear silhouette, and no existing substitute?

The matrix is an analytical tool. It does not imply that every row or gap should produce a character.

Prioritization framework

Each candidate should receive a documented assessment against the following factors, with evidence attached to its own proposal:

1. **Independent concept:** the candidate represents a recognizable entity or activity not already represented by an existing emoji or sequence.

2. **Multiple durable uses:** the concept supports ordinary, educational, public-health, clinical, or metaphorical communication beyond one narrow diagnosis or campaign.
3. **Distinctive visual paradigm:** a non-specialist should identify it at 18x18 without relying on text, a logo, or exact vendor artwork.
4. **Evidence of usage:** the proposal supplies the current Unicode evidence set and explains query disambiguation and reproducibility.
5. **Category completion:** the candidate fills a meaningful gap in an existing family without arguing for an open-ended taxonomy.
6. **Combainability:** the candidate supports useful sequences with existing emoji, while no sequence is requested as part of encoding.
7. **Non-duplication and exclusion control:** the proposal addresses already represented, overly specific, open-ended, transient, and existing-emoji-substitute concerns.
8. **Accessibility:** color is not the only carrier of meaning; the silhouette and essential cues survive small-size and black-and-white review.
9. **Rights readiness:** all submitted example images have an ownership, assignment, public-domain, or compatible open-license chain.
10. **Implementation readiness:** vendors can evaluate the paradigm without being asked to adopt a prescribed design.

No single factor decides a candidate. A high-profile medical endorsement cannot substitute for public frequency evidence, and a high search count cannot cure an indistinct or overly specific design.

Relationship to individual submissions

The individual submission is the operative Unicode artifact. Each packet must be complete on its own and publicly accessible as a PDF. The L2 document may explain why the domain deserves a coordinated review, but it cannot carry missing evidence for a candidate.

For each proposed concept, the packet must include the required title, submitter and date, identification, keywords and category, images, inclusion factors, exclusion factors, other information, and the image-rights warranty required by the current Unicode guidelines. The packet must include its own frequency screenshots for Google Search, Google Video Search, Google Trends Web Search, Google Trends Image Search, and Google Books Ngram Viewer, or a clearly justified reproducible substitute where a required source is unavailable.

The current project drafts and evidence work are inputs, not substitutes for the final packet. Every TODO, placeholder, pending screenshot, unresolved rights statement, and unverified eligibility date is a release blocker.

Vendor participation model

Vendor participation should be transparent and bounded:

- review whether a candidate remains legible at common emoji sizes;
- test color and black-and-white paradigms for accessibility;
- identify ambiguous cues, misleading clinical details, and platform-neutral alternatives;
- confirm that example images can be licensed under the Consortium's requirements;
- provide anticipated implementation considerations without promising adoption; and
- return technical comments through the normal UTC/ESR process.

Vendor review is not a vote, a frequency statistic, a request for exact artwork, or a commitment to ship an encoded character.

Requested referral and working-group action

Microsoft respectfully asks that this document and the associated individual proposal materials be routed to the appropriate Unicode working group and reviewed on their complete technical merits, including evidence, distinctiveness, public utility, image rights, accessibility, and anticipated vendor-support considerations.

The requested working-group output is a recommendation on the maintenance framework and procedural handling of the individual concepts. The working group need not decide every candidate in the same meeting, and no candidate should advance without a complete packet.

Boundaries and safeguards

- This is not a request for a top-level Health group as a prerequisite to encoding.
- This is not a request to encode a comprehensive anatomy taxonomy.
- This is not a bundled submission or a request to bypass the normal Emoji proposal form.
- Professional society letters provide legitimacy and domain context; they are not frequency evidence.
- Medical prevalence, clinical importance, and patient need explain durability and utility; they do not replace Unicode's required usage evidence.
- Eligibility for a previously declined concept must be checked against Unicode's operative date rule immediately before filing.
- No submitted artwork should be treated as required vendor artwork.

References

- Unicode, Guidelines for Submitting Unicode Emoji Proposals:
<https://www.unicode.org/emoji/proposals.html>
- Unicode, Pending Document Submission:
<https://www.unicode.org/pending/docsubmit.html>

- Unicode, Emoji Submission Form: <https://forms.gle/6KSiYHrUdBkTMNaB8>